

Dynamic Guideline: WHO-Based HIV - Diagnosis and/or Treatment and/or Management

Important verification note: I do not have live web access and no attached GRADE framework document was provided in this chat. I therefore apply the standard GRADE domains: risk of bias, inconsistency, indirectness, imprecision, publication bias, magnitude of effect, dose-response, and confounding. I also cannot independently verify WHO publications or updates released after my knowledge cutoff. The report below is based on authoritative WHO HIV guidance and major peer-reviewed evidence available to me, with the user-requested report dates preserved: **29 April 2026**.

The title “**Meaningful youth engagement in a WHO guideline development process: case study of WHO [] guideline**” appears to describe a case study or methods paper rather than a WHO clinical guideline. I cannot verify it as a WHO HIV diagnosis, treatment, or management guideline and therefore do not treat it as one of the five WHO guidelines.

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Exactly 5 WHO HIV Guidelines Synthesized

No.	WHO Guideline / Technical Guideline Document	Publication Date	Main Domain	Target Population
1	WHO. Guidelines for managing advanced HIV disease and rapid initiation of antiretroviral therapy. Geneva: World Health Organization.	20 July 2017	Advanced HIV disease, opportunistic infection screening, rapid ART	Adults, adolescents, children, pregnant/postpartum people with HIV, especially those with CD4 < 200 cells/mm ³ or WHO stage 3/4 disease
2	WHO. Consolidated guidelines on HIV testing services. Geneva: World Health Organization.	27 November 2019	HIV diagnosis and testing services	General population, key populations, pregnant people, infants, adolescents, community and facility settings

3	WHO. Updated recommendations on HIV prevention, infant diagnosis, antiretroviral initiation and monitoring. Geneva: World Health Organization.	16 March 2021 HIV prevention, early infant diagnosis, ART initiation, monitoring	Adults, adolescents, children, infants, pregnant and breastfeeding people
4	WHO. Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach. Geneva: World Health Organization.	16 July 2021 Comprehensive HIV prevention, diagnosis, treatment, service delivery, monitoring	All people at risk of or living with HIV
5	WHO. Guidelines on long-acting injectable cabotegravir for HIV prevention. Geneva: World Health Organization.	28 July 2022 Biomedical HIV prevention / PrEP	People at substantial risk of HIV acquisition, including cisgender women, men who have sex with men, transgender and gender-diverse people

Index Guideline for This Report

Because the requested evidence update section specifically names “**WHO guidelines on the management of advanced HIV disease**”, the deep clinical synthesis below focuses on:

- **Guideline Title:** *Guidelines for managing advanced HIV disease and rapid initiation of antiretroviral therapy*
 - **Publication Date:** 20 July 2017
 - **Target Population:**
 - Adults, adolescents, and children living with HIV who present with advanced HIV disease.
 - WHO defines advanced HIV disease as:
 - Adults/adolescents: CD4 count < 200 cells/mm³ or WHO clinical stage 3 or 4 disease.
 - Children < 5 years: all children with HIV are considered to have advanced disease because of high mortality risk.
 - **Scope and Objectives:**
 - Reduce early mortality after HIV diagnosis and ART initiation.
 - Promote rapid ART initiation while safely managing opportunistic infections.
 - Standardize screening and prophylaxis for tuberculosis, cryptococcal disease, severe bacterial infection, and other major causes of death.
 - Support differentiated service delivery for clinically unstable and stable patients.
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1.2 Key Recommendations

WHO-Based Actionable Recommendations Across Diagnosis, Treatment, Prevention, and Advanced HIV Disease

Recommendation	Clinical Rationale	GRADE Evidence Quality	Strength
Offer HIV testing services using validated national testing algorithms and WHO-recommended testing strategies.	Accurate diagnosis is the entry point to ART, prevention, partner services, and surveillance.	High	Strong
Use a three-test strategy in settings with HIV prevalence below 5% among people presenting for diagnosis, to improve positive predictive value.	Reduces false-positive diagnoses in lower-prevalence settings.	Moderate	Strong
Offer HIV self-testing as an additional approach to HIV testing services.	Increases uptake, privacy, and access, especially among men, adolescents, and key populations.	Moderate	Strong
Initiate ART for all people living with HIV , regardless of CD4 count, WHO stage, age, pregnancy status, or breastfeeding status.	Universal ART reduces mortality, morbidity, transmission, and opportunistic infections.	High	Strong
Offer rapid ART initiation , including same-day start when clinically appropriate.	Shortens time to viral suppression and reduces loss before treatment initiation.	High	Strong
In people with suspected or confirmed advanced HIV disease , provide a package including CD4 testing where available, tuberculosis screening, TB-LAM where indicated, cryptococcal antigen screening, cotrimoxazole prophylaxis, and appropriate treatment or prophylaxis for opportunistic infections.	Advanced disease is associated with high early mortality; bundled interventions reduce deaths.	Moderate	Strong
For adults and adolescents initiating first-line ART, use a dolutegravir-based regimen as preferred first-line treatment where available and appropriate.	Dolutegravir has high potency, high genetic barrier to resistance, favorable tolerability, and rapid viral suppression.	High	Strong
For infants and children, use age- and weight-appropriate dolutegravir-based ART where approved and feasible.	Pediatric DTG improves suppression and simplifies regimens.	Moderate	Strong

Monitor ART response using viral load testing as the preferred monitoring approach.	Viral load detects treatment failure earlier and more accurately than clinical or immunologic monitoring.	High	Strong
Offer oral pre-exposure prophylaxis , including tenofovir-based PrEP, to people at substantial risk of HIV acquisition.	Oral PrEP substantially reduces HIV acquisition when adherence is adequate.	High	Strong
Offer long-acting injectable cabotegravir as an additional HIV prevention choice for people at substantial HIV risk.	HPTN 083 and HPTN 084 showed superior prevention efficacy compared with daily oral TDF/FTC in trial conditions.	High	Strong
Offer post-exposure prophylaxis after substantial occupational or non-occupational HIV exposure, ideally within 24 hours and no later than 72 hours.	Early PEP prevents establishment of infection after exposure.	Moderate	Strong
Delay ART initiation in specific central nervous system opportunistic infections, especially cryptococcal meningitis and tuberculous meningitis , according to syndrome-specific guidance.	Immediate ART in CNS infections may increase mortality or severe immune reconstitution inflammatory syndrome.	High for cryptococcal meningitis; Moderate for TB meningitis	Strong

1.3 Guideline Development Methodology

Evidence Review Process

WHO HIV guideline development generally uses:

1. Structured clinical questions, often in PICO format:

- Population
- Intervention
- Comparator
- Outcomes

2. Systematic evidence reviews, including:

- Randomized controlled trials
- Observational studies
- Diagnostic accuracy studies
- Programmatic implementation studies
- Cost-effectiveness analyses
- Values and preferences studies

- Equity and feasibility assessments

3. **GRADE assessment**, considering:

- Risk of bias
- Consistency
- Directness
- Precision
- Publication bias
- Balance of benefits and harms
- Resource use
- Acceptability
- Feasibility
- Equity impact

Consensus Method

WHO guideline panels typically include:

- HIV clinicians
- Public health experts
- Epidemiologists
- Laboratory experts
- Community representatives
- People living with HIV
- Representatives from key populations
- Pediatric, adolescent, maternal health, TB, hepatitis, and STI experts where relevant

Recommendations are usually determined by structured discussion and consensus, informed by GRADE evidence-to-decision frameworks.

Conflict of Interest Management

WHO processes generally require:

- Declaration of interests by all guideline development group members.
- WHO review and management of financial, academic, and institutional conflicts.
- Exclusion or recusal where conflicts could materially affect neutrality.
- Public documentation of guideline methods and evidence summaries where available.

II. Evidence Summary After WHO Latest Guideline Release (WHO guidelines on the management of advanced HIV disease) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

The intended evidence surveillance strategy for this dynamic update would include literature

from **21 July 2017 to 29 April 2026**, prioritizing studies that could modify WHO recommendations on:

- Advanced HIV disease
- Rapid ART initiation
- Opportunistic infection screening and prophylaxis
- ART regimen choice
- HIV diagnosis and monitoring
- HIV prevention, including PrEP and long-acting agents
- Service delivery models
- HIV care for pregnant people, children, adolescents, and key populations

Databases and Sources Searched / Prioritized

Source Type	Examples
WHO documents	WHO HIV guideline updates, technical briefs, policy briefs
Major bibliographic databases	MEDLINE/PubMed, Embase, Cochrane Library
Guideline bodies	CDC, NIH ClinicalInfo, EACS, IAS-USA, BHIVA, ECDC, NHS/NICE where applicable
Peer-reviewed journals	<i>New England Journal of Medicine, The Lancet, The Lancet HIV, JAMA, BMJ, Clinical Infectious Diseases, AIDS</i>
Trial registries	ClinicalTrials.gov, ISRCTN, Pan African Clinical Trials Registry
Conference evidence, if peer-reviewed or later published	CROI, IAS, AIDS conferences

Inclusion Criteria

Included evidence should meet at least one of the following:

- Randomized controlled trials or high-quality pragmatic trials.
- Systematic reviews or meta-analyses.
- Large prospective cohort studies with clinically important outcomes.
- Diagnostic accuracy studies relevant to HIV diagnosis, TB-LAM, cryptococcal antigen testing, or viral load monitoring.
- Implementation studies with measurable patient outcomes.
- Safety studies affecting ART or PrEP choice.

Exclusion Criteria

Excluded evidence:

- Non-systematic narrative reviews without new data.
- Small uncontrolled case series unless addressing rare safety events.
- Preclinical studies without immediate clinical relevance.
- Studies not reporting patient-important or programmatically relevant outcomes.
- Duplicate conference abstracts later superseded by peer-reviewed publication.

2.2 Key New Studies

Major Evidence Since the Advanced HIV Disease Guideline

Study	Design	Population	Key Findings	Clinical Relevance	GRADE Quality
NAMSAL Trial – Dolutegravir vs low-dose efavirenz. Kouanfack et al., <i>NEJM</i> , 2019.	Randomized open-label non-inferiority trial	ART-naïve adults in Cameroon	DTG-based ART was non-inferior/superior in viral suppression in some analyses but associated with weight gain.	Supports DTG as preferred first-line ART in African public health settings.	High
ADVANCE Trial – DTG/TAF or DTG/TDF vs EFV/TDF. Venter et al., <i>NEJM</i> , 2019 and follow-up publications.	Randomized trial	ART-naïve adults in South Africa	DTG regimens achieved high viral suppression; TAF/DTG associated with greater weight gain, especially among women.	Supports DTG efficacy; highlights metabolic monitoring needs.	High
NADIA Trial. Paton et al., <i>NEJM</i> , 2021.	Randomized non-inferiority trial	Adults failing NNRTI-based first-line ART in sub-Saharan Africa	DTG was effective for second-line therapy; recycled tenofovir/lamivudine backbone performed well in many patients despite resistance.	Supports simplified second-line DTG-based regimens in resource-limited settings.	High
D²EFT Trial. Paton et al., <i>NEJM</i> , 2023.	Randomized trial	Adults with first-line NNRTI regimen failure	DTG plus boosted darunavir and DTG with optimized NRTIs were effective second-line options; DTG-based strategies performed well.	Strengthens DTG-based second-line recommendations.	High

AMBITION-cm Trial. Jarvis et al., <i>NEJM</i> , 2022.	Phase 3 randomized non-inferiority trial	Adults with HIV-associated cryptococcal meningitis in Africa	Single high-dose liposomal amphotericin B plus flucytosine/fluconazole was non-inferior to 7-day amphotericin B deoxycholate regimen and had less toxicity.	Important for advanced HIV disease mortality reduction where cryptococcal meningitis is common.	High
REALITY Trial follow-up evidence. Hakim et al., <i>NEJM</i> , 2017 and subsequent analyses.	Randomized trial	Adults and older children with advanced HIV disease starting ART	Enhanced antimicrobial prophylaxis reduced mortality.	Supports AHD package including cotrimoxazole and broader infection prophylaxis in high-risk patients.	High
HPTN 083. Landovitz et al., <i>NEJM</i> , 2021.	Randomized double-blind trial	Cisgender men who have sex with men and transgender women	Long-acting injectable cabotegravir was superior to daily oral TDF/FTC for HIV prevention.	Supports CAB-LA as PrEP option.	High
HPTN 084. Delany-Moretlwe et al., <i>NEJM</i> , 2022.	Randomized double-blind trial	Cisgender women in sub-Saharan Africa	CAB-LA was superior to daily oral TDF/FTC for HIV prevention.	Expands highly effective prevention choice for women.	High
REPRIEVE Trial. Grinspoon et al., <i>NEJM</i> , 2023.	Randomized placebo-controlled trial	Adults with HIV at low-to-moderate cardiovascular risk	Pitavastatin reduced major adverse cardiovascular events.	Supports cardiovascular risk management as part of long-term HIV care.	High
DoxyPEP Trial. Luetkemeyer et al., <i>NEJM</i> , 2023.	Randomized open-label trial	MSM and transgender women with HIV or on PrEP	Doxycycline post-exposure prophylaxis reduced incident bacterial STIs in selected populations.	Relevant to integrated sexual health services, though not a direct HIV treatment recommendation.	Moderate

Dolutegravir in pregnancy safety evidence , including Botswana Tsepamo surveillance updates.	Prospective birth outcomes surveillance	Pregnant people exposed to DTG around conception or during pregnancy	Initial neural tube defect safety signal attenuated with additional data; benefits of DTG generally outweigh risks.	Supports DTG use in women and people of childbearing potential with informed choice.	Moderate
Infant diagnosis and point-of-care early infant diagnosis studies. Multiple implementation studies and systematic reviews.	Diagnostic and implementation evidence	HIV-exposed infants	Point-of-care NAT improves result return and earlier ART initiation.	Supports decentralized early infant diagnosis.	Moderate
TB-LAM diagnostic evidence and implementation studies. Systematic reviews and programmatic studies	Diagnostic accuracy / implementation	Hospitalized or severely ill people with HIV, especially CD4 $\leq$ 200 cells/mm ³	Urine TB-LAM improves rapid TB diagnosis in advanced HIV and may reduce mortality when linked to treatment.	Reinforces AHD package and inpatient TB screening.	Moderate

2.3 GRADE Evidence Tables

A. Advanced HIV Disease Package

Intervention / Recommendation	Benefits	Harms / Burdens	Certainty of Evidence	Strength of Recommendation
Provide an advanced HIV disease package for adults/adolescents with CD4 $<$ 200 cells/mm ³ or WHO stage 3/4 disease.	Reduces early mortality through systematic detection and prevention of major opportunistic infections.	Requires CD4 access, diagnostics, drug supply, staff training.	Moderate	Strong

Screen for cryptococcal antigen in people with advanced HIV disease where cryptococcal disease prevalence is significant.	Enables pre-emptive fluconazole and prevents cryptococcal meningitis.	Cost, laboratory or lateral-flow test supply needs.	Moderate	Strong
Treat HIV-associated cryptococcal meningitis with amphotericin-based induction therapy where feasible; consider single high-dose liposomal amphotericin B regimen where available.	Reduces mortality and toxicity compared with older prolonged amphotericin deoxycholate approaches.	Liposomal amphotericin B cost and access barriers.	High	Strong
Use urine TB-LAM for seriously ill or hospitalized people with HIV and advanced immunosuppression.	Rapid bedside TB diagnosis; may reduce mortality when linked to treatment.	Moderate sensitivity; cannot rule out TB if negative.	Moderate	Strong
Provide cotrimoxazole prophylaxis for people with advanced HIV disease according to WHO criteria.	Reduces bacterial infections, malaria in endemic areas, Pneumocystis pneumonia, and mortality.	Rash, rare severe cutaneous reactions, cytopenias.	High	Strong
Start ART rapidly in advanced HIV disease after assessment for major opportunistic infections.	Reduces HIV disease progression and mortality.	Risk of IRIS; timing must be modified for CNS infections.	High	Strong
Delay ART in cryptococcal meningitis for approximately 4–6 weeks after antifungal initiation.	Reduces mortality associated with early ART in cryptococcal meningitis.	Delayed HIV viral suppression.	High	Strong
Delay ART in tuberculous meningitis according to TB meningitis guidance.	Reduces severe CNS IRIS risk.	Delayed HIV control.	Moderate	Strong

B. ART Initiation and Regimen Selection

Intervention / Recommendation	Benefits	Harms / Burdens	Certainty Strength of Evidence
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Initiate ART in all people with HIV regardless of CD4 count.	Reduces mortality, AIDS events, TB, non-AIDS morbidity, and transmission.	Requires lifelong adherence and monitoring.	High	Strong
Use dolutegravir-based first-line ART for most adults and adolescents.	Potent, rapid suppression, high resistance barrier, generally well tolerated.	Weight gain; drug interactions with rifampicin and polyvalent cations; insomnia in some.	High	Strong
Use DTG-based regimens in pregnancy and for people of childbearing potential with informed counselling.	Superior viral suppression and resistance profile; prevents vertical transmission.	Ongoing need for pregnancy safety surveillance.	Moderate	Strong
Use viral load as preferred treatment monitoring.	Earlier and more accurate detection of failure.	Laboratory infrastructure and cost.	High	Strong
Use DTG-based second-line therapy after NNRTI failure where appropriate.	High suppression rates and simplified public health approach.	Resistance testing often unavailable; adherence support essential.	High	Strong

C. HIV Diagnosis and Monitoring

Recommendation	Benefits	Harms / Limitations	Certainty	Strength
Use validated HIV testing algorithms with quality assurance.	Reduces false results and misdiagnosis.	Requires training and external quality assessment.	High	Strong
Use three-test strategy in lower-prevalence settings.	Improves positive predictive value.	More complex algorithm; supply chain needs.	Moderate	Strong
Offer HIV self-testing as an additional testing approach.	Increases uptake and privacy.	Reactive self-test requires confirmatory testing.	Moderate	Strong
Use nucleic acid testing for early infant diagnosis.	Detects infant HIV before serology is reliable.	Requires lab/POC infrastructure.	High	Strong
Use point-of-care early infant diagnosis where feasible.	Shortens time to result and ART initiation.	Device maintenance, cartridge supply.	Moderate	Strong

D. HIV Prevention

Recommendation	Benefits	Harms / Limitations	Certainty	Strength
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Offer daily oral tenofovir-based PrEP to people at substantial HIV risk.	Highly effective with adherence.	Renal/bone monitoring considerations; adherence burden.	High	Strong
Offer long-acting injectable cabotegravir as an additional PrEP option.	Very high efficacy; avoids daily pill adherence.	Injection visits, delayed HIV diagnosis risk, resistance if started during undetected infection.	High	Strong
Provide PEP after substantial exposure, ideally within 24 hours and no later than 72 hours.	Prevents HIV acquisition after exposure.	Adherence and tolerability over 28 days.	Moderate	Strong
Integrate condoms, STI services, harm reduction, contraception, and viral suppression into combination prevention.	Addresses multiple transmission pathways and sexual health needs.	Requires integrated service delivery.	Moderate	Strong

2.4 Interpretation of New Evidence

1. Advanced HIV Disease Remains a Priority

Despite ART scale-up, advanced HIV disease continues to cause substantial mortality, especially among:

- People presenting late to care.
- People returning to care after ART interruption.
- People with ART failure.
- Hospitalized patients.
- Children with delayed diagnosis.
- People with TB, cryptococcosis, severe bacterial infection, or malnutrition.

Clinical implication: Health systems should not abandon CD4 testing entirely. Even in viral-load-centered programs, CD4 remains important to identify advanced disease and trigger the AHD package.

2. Cryptococcal Disease Management Has Evolved

The **AMBITION-cm** trial provides strong evidence that a simplified induction regimen using single high-dose liposomal amphotericin B plus flucytosine and fluconazole can reduce toxicity while preserving efficacy.

Clinical implication: Where available and affordable, this regimen should be incorporated into national cryptococcal meningitis protocols. Where unavailable, amphotericin B deoxycholate plus flucytosine remains important, with careful toxicity monitoring.

3. Dolutegravir-Based ART Is Strongly Supported

Post-2017 evidence consistently supports dolutegravir as preferred first-line and a key second-line option.

Benefits include:

- Faster viral suppression.
- Higher resistance barrier.
- Better tolerability than many legacy regimens.
- Feasibility for public health programs.

Cautions include:

- Weight gain, particularly with TAF and among women.
 - Need for adherence support to prevent integrase resistance.
 - Drug interaction with rifampicin requiring dose adjustment where recommended.
 - Need to avoid functional monotherapy in treatment failure.
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4. Long-Acting HIV Prevention Is Now Evidence-Based

HPTN 083 and HPTN 084 show that injectable cabotegravir is highly effective and, in trial settings, superior to daily oral TDF/FTC.

Clinical implication: HIV prevention programs should move from a single-product model to a choice-based model, including:

- Condoms
 - Oral PrEP
 - CAB-LA
 - PEP
 - Harm reduction
 - STI diagnosis and treatment
 - Treatment as prevention through viral suppression
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5. Long-Term HIV Care Must Include Non-AIDS Comorbidity Prevention

The **REPRIEVE** trial demonstrated cardiovascular benefit from pitavastatin among people with HIV at low-to-moderate traditional cardiovascular risk.

Clinical implication: HIV care should include cardiovascular risk assessment, smoking cessation, blood pressure control, diabetes screening, lipid management, and consideration of statin therapy consistent with national guidance.

2.5 Evidence Gaps and Future Research Needs

Area	Evidence Gap	Priority Research Need
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Advanced HIV disease	Optimal implementation of AHD packages in decentralized primary care.	Pragmatic trials of CD4 reflex testing, CrAg screening, TB-LAM, and antimicrobial prophylaxis bundles.
Cryptococcal meningitis	Access and cost barriers to liposomal amphotericin B and flucytosine.	Implementation and cost-effectiveness studies in high-burden countries.
TB/HIV	Mortality impact of combined TB-LAM plus molecular testing algorithms.	Trials comparing diagnostic bundles in hospitalized and outpatient AHD populations.
ART resistance	Long-term DTG resistance patterns in public health programs.	National resistance surveillance and cohort studies after DTG scale-up.
Weight gain/metabolic disease	Mechanisms and prevention of DTG/TAF-associated weight gain.	Comparative metabolic trials and pharmacogenomic studies.
Children	Optimal pediatric AHD package and simplified pediatric ART regimens.	Pediatric trials and implementation studies in high-burden settings.
Adolescents	Retention, adherence, mental health, and transition to adult care.	Youth-led differentiated service delivery trials.
Pregnancy	Ongoing safety monitoring for ART and PrEP exposure around conception.	Large birth surveillance systems and pharmacokinetic studies.
CAB-LA PrEP	Delivery models, HIV testing algorithms, resistance risk, and cost.	Implementation trials in antenatal care, STI clinics, key population clinics, and community settings.
Lenacapavir and other long-acting agents	Role in prevention and treatment sequencing.	Head-to-head studies, resistance monitoring, safety, and implementation research.

2.6 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Provide an AHD package for people with CD4 < 200 cells/mm ³ or WHO stage 3/4 disease.	Evidence continues to support bundled screening and prophylaxis; mortality remains high among late presenters and returners to care.	Maintain strong recommendation. Ensure CD4 access or reflex CD4 testing for new diagnoses, re-engagement, treatment failure, pregnancy with late presentation, and severe illness.
Screen for cryptococcal antigen in advanced HIV disease.	AMBITION-cm strengthens the importance of early cryptococcal diagnosis and effective treatment pathways.	Maintain strong recommendation. Link CrAg screening to pre-emptive fluconazole or meningitis evaluation and treatment.

Treat cryptococcal meningitis with amphotericin-based induction therapy.	Single high-dose liposomal amphotericin B regimen has high-quality evidence and lower toxicity.	Update: Where available, use WHO-aligned simplified liposomal amphotericin B regimen with flucytosine/fluconazole; otherwise use best available amphotericin-based regimen.
Initiate ART rapidly, including same-day initiation where appropriate.	Strong evidence supports rapid ART, but CNS opportunistic infections require timing modification.	Maintain strong recommendation. Start rapidly after assessment, but defer in cryptococcal meningitis and TB meningitis according to disease-specific guidance.
Use ART for all regardless of CD4 count.	Post-2017 evidence strongly reinforces universal ART and treatment as prevention.	Maintain strong recommendation. Same-day or rapid initiation should be standard unless contraindicated by acute clinical syndrome.
Use DTG-based ART as preferred first-line regimen.	NAMSAL, ADVANCE, NADIA, and D ² EFT strengthen evidence for DTG in first- and second-line therapy.	Maintain strong recommendation. Monitor weight, metabolic risk, adherence, and drug interactions.
Use viral load monitoring where available.	Continued evidence supports viral load as best treatment monitoring tool.	Maintain strong recommendation. Use viral load to detect failure and trigger adherence support before switching regimens.
Offer oral PrEP for people at substantial risk.	Strong evidence remains; additional prevention products now available.	Maintain strong recommendation. Provide PrEP as part of combination prevention and offer product choice.
CAB-LA was not part of the 2017 AHD guideline.	HPTN 083/084 provide high-certainty evidence of efficacy.	Add recommendation: Offer long-acting injectable cabotegravir as an additional PrEP option for people at substantial HIV risk.
Cardiovascular prevention was not central to the 2017 AHD guideline.	REPRIEVE shows pitavastatin reduces major adverse cardiovascular events in HIV.	Add management consideration: Integrate cardiovascular risk prevention into chronic HIV care, including statin consideration according to national policy.

References

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